cohere HEALTH

Cohere Health's Response to CMS Request for Information: Health Technology Ecosystem

Submitted by: Cohere Health

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Introduction

Cohere Health is a patient-centric company committed to streamlining the clinical decision-making processes for health insurers and physicians, with the ultimate aim of improving patient care and health outcomes. Launched in 2019, Cohere is transforming prior authorization–a process that presents numerous financial, administrative, and clinical concerns for all stakeholders.

Our mission directly aligns with the Administration's policy to substantially reduce private expenditures associated with Federal regulation compliance, particularly as it relates to the process of prior authorization (PA), achieved through a focus on intelligent automation, interoperability, and the implementation of evidence-based care pathways. Enclosed in our RFI response is an outline of how we're unleashing innovation and reducing burden in Medicare through a new approach to PA:

- **Applying innovative technology to PA** can provide needed oversight and transparency of health insurance for Americans; **minimize physician burnout; improve patients' health outcomes; and limit inappropriate care, waste, and abuse** in our nation's healthcare system.
- Using the latest developments in artificial intelligence (AI) and machine learning (ML) responsibly can surface **real-time approvals for patient care,** while ensuring care is never denied by AI/technology. This approach, coupled with an **improved peer-to-peer process** between physicians of the same specialty, will ensure patient safety, improve clinical outcomes, and significantly optimize the healthcare system.
- Cohere has observed an **80 percent reduction in care delays**, a **40 percent reduction in healthcare providers time spent on authorizations**, and a **significant reduction in denials** of patient care using intelligent PA. This strategy is preferred by providers, as evidenced by our **93 percent provider satisfaction rating**.

Our close work with Medicare Advantage (MA) plans, as well as Commercial and Managed Medicaid plans, makes us a valuable thought partner to the agency. We have tested and optimized best practices for PA that have the potential to truly transform the system. We would also be thrilled to help CMS pilot its first-ever AI-driven PA solution within the Medicare Fee-for-Service (FFS) program, leveraging the innovation afforded by the private sector. Adopting an approach for Medicare FFS that more closely resembles the approach in MA would be an effective way for CMS to capture savings for the program, identify inappropriateness and abuse patterns across the delivery system, and ensure patient safety and appropriate clinical care. This will significantly reduce confusion and abrasion amongst providers who submit for both MA and FFS. Cohere could identify a subset of codes to pilot this approach with the Medicare Administrative Contractors (MACs).

1. Patient Needs

Intelligent PA is an important mechanism to ensure patients receive safe and appropriate medical care without unnecessary delays. Patient safety is of utmost importance and care decisions must use evidence-based criteria. For example, Cohere's ML models can identify retropulsion *(bone fragments pushing into spinal canal)* on an MRI report, which is a contraindication for a kyphoplasty *(injected cement into the vertebral body to treat a compression fracture)*. If the technology determines that the request is not appropriate for auto-approval, it immediately pends the case for physician review.

Some treatments, such as spinal cord stimulators, are invasive and have unpredictable and variable outcomes. It is vital to carefully look for all indications and ensure all clinical criteria are met before rendering an approval. Cohere has found that spinal cord stimulators are grossly overused; only 50 percent of patients who receive a spinal cord stimulator have positive outcomes. This aligns with the evidence that this should be a treatment of last resort for patients with spinal issues. If any gaps in the clinical record indicate a patient may not meet all the criteria for appropriateness and effectiveness based on the guidelines, an authorization requires clinician review.

Peer-to-peer consultations can be extremely beneficial for ensuring appropriate and *safe* clinical care, so long as the conversation is held between physicians of the same specialty (or, ideally, sub-specialty). Oftentimes, peer-to-peer reviews can be regarded as adversarial or burdensome for physicians, but there is untapped potential in leveraging this process as more akin to second opinion/consults, which Medicare regulations do not support today. Cohere encourages CMS to require that peer-to-peers take place between physicians of the same specialty to improve patient safety. Leveraging peer-to-peer consultations as a second opinion process would also ensure that the latest and greatest treatments, clinical protocols, and evidence can be applied to patients, even in the most remote and rural settings, so that no matter where a patient lives, they too receive access to evidence-based medicine and safe, quality outcomes.

Cohere appreciated Dr. Oz's comments on limiting the code set to reduce the burden on physicians. We believe that the number of codes is not the relevant issue, however, and that the area for greater opportunity and impact is around the scope of care appropriateness. We believe the best way to reduce provider abrasion is to focus oversight on where in the code set inappropriate use most frequently occurs. Cohere has the data that can help CMS optimize which codes should be on a PA list–codes that are and are not inappropriately used–to help with list optimization.

2. Data Access and Integration

PC–8. In your experience, what health data is readily available and valuable to patients or their caregivers or both?

- Readily Available (to varying degrees): Basic demographics, appointment schedules, medication lists, recent lab results.
- Valuable:
    - Prior Authorization Status and Clinical Criteria: Highly valuable for patients and caregivers to understand the progression of their requested care, whether available digitally or via conversation with their physician.
    - Comprehensive Clinical History: Including past diagnoses, treatments, allergies, and family medical history.
    - Imaging Reports and Images (e.g., X-rays, MRIs): Often crucial for understanding diagnoses and treatment plans.

    - Specialist Consultation Notes: Providing detailed insights from different medical perspectives.
    - Referral Information: Status, receiving provider details.
    - Out-of-Pocket Cost Estimates: Binding cost estimates for defined periods are invaluable for financial planning.
    - Provider Quality: Assist patients in choosing the best physician and facility for their care.

a. What data is valuable, but hard for patients and caregivers, or app developers and other technical vendors, to access for appropriate and valuable use (for example, claims data, clinical data, encounter notes, operative reports, appointment schedules, prices)?

- Prior Authorization Data: This is a prime example. While patients know a prior authorization is required, accessing its real-time status, the specific clinical criteria being applied, or the reasons for a denial is extremely difficult, leading to significant care delays and abrasion. Cohere Health delivers real-time prior authorization decisions so physicians can provide their patients with this critical information.
- Comprehensive Clinical Data across disparate providers: Aggregating data from multiple EHRs (especially scanned documents, faxed records, and free-text notes) is a significant challenge due to lack of interoperability and patient data/information blocking.
- Binding Cost Estimates and Payer-Specific Price Information: These are often opaque and difficult for patients to obtain upfront.
- Full Claims Data: While Blue Button 2.0 provides some data, granular claims data linked to clinical interventions and outcomes is hard to access for actionable insights, particularly due to the retrospective nature and time lag of medical claims.
- Operative Reports and Detailed Procedure Notes: Crucial for understanding complex interventions.
- Longitudinal Patient Journeys: Understanding the full sequence of care for a given condition, often fragmented across multiple systems. At the core of Cohere's technology are longitudinal care paths, along which prior authorization decisions are made. This empowers physicians to have care planning and decision-making conversations with their patients.
- Accurate Information on Providers, Their Practice Affiliation & Cost of Services: This data is often fragmented and out of date, which creates confusion in ensuring patients are able to find appropriate care with providers in a cost effective manner.

b. What are specific sources, other than claims and clinical data, that would be of highest value, and why?

- Prior Authorization Decision Data: Prior authorization data is some of the most valuable health care information that can be leveraged for population health and care planning purposes. The insights Cohere Health gleans from our intelligent prior authorization platform can be leveraged for evidence-based, AI-driven clinical decision-making. When a prior authorization is approved or denied, the structured data on the criteria used, the clinical rationale, and the specific codes involved provides invaluable insights into appropriate care pathways and can identify potential areas of waste or inefficiency. Applying AI and predictive analytics to a prior authorization request can render valuable signals for patient care planning or broader population health strategies.
- Clinical Guidelines - Medical Necessity Criteria: Standardized, machine-readable versions of the exact clinical guidelines used by each payer for prior authorization decisions. This would significantly reduce provider abrasion and help patients understand the medical necessity criteria. The current landscape of clinical guideline development involves content organizations that

essentially have a duopoly in the market. This fragmentation creates a significant administrative burden for providers who must navigate sometimes conflicting or outdated guidelines when submitting authorizations for services, leading to inconsistencies in medical necessity determinations and increased appeals. We have also seen, in certain cases, that the evidence-base evolves faster than NCDs/LCDs are updated, and that not all states have LCD guidance.

- Clinical Outcomes and Patient-Reported Outcome Measures (PROMs): Clinical outcomes data reporting patient status, progress, diagnoses, etc. as well as data reported directly from patients about their symptoms, functional status, and quality of life. This adds the patient's voice to clinical data.
- Social Determinants of Health (SDOH) Data: Information on factors like housing, food security, and transportation that significantly impact health outcomes.
- Provider Network and Quality Data: More granular data on provider specialties, certifications, referral patterns, experience with specific conditions, and actual quality metrics (beyond basic directories) would be highly valuable for care navigation and finding the "right" provider. This data can also be leveraged to build high-value/quality specialty referral networks.

c. What specific opportunities and challenges exist to improve accessibility, interoperability and integration of clinical data from different sources to enable more meaningful clinical research and generation of actionable evidence?

- Opportunities
    - Mandatory FHIR API Adoption: Universal adoption and robust implementation of FHIR APIs for all data exchange, particularly the Prior Authorization API, will unlock significant potential.
    - AI/ML for Data Normalization and Extraction: Advanced language models and machine learning can help extract structured data from unstructured formats (e.g., free-text notes, scanned documents) and normalize data from disparate sources, making it more usable for clinical decision support and prior authorization as Cohere does, or for other applications like clinical research.
    - Federated Data Models: Enabling data to remain at its source while allowing queries across a network, preserving privacy while enabling broader research.
    - Integrated Standard Clinical Ontologies: Move from disparate coding systems (e.g., ICD, CPT) to an integrated approach with unified relationship definitions can accelerate ability to derive key insights and accelerate AI/ML solution development
    - Increased Investment in Public Health Data Infrastructure: Strengthening public health data systems can improve population-level insights.
- Challenges:
    - Patient Data/Information Blocking: Remains a primary impediment, as previously highlighted.
    - Data Silos and Proprietary Systems: EHR vendors and legacy systems often lock data into proprietary formats, hindering seamless exchange.
    - Lack of Data Standardization: Inconsistent terminology, coding practices, relationship mappings, and data formats across providers and systems.
    - Privacy and Security Concerns: Ensuring robust protection of sensitive patient health information during exchange and aggregation.
    - Cost of Interoperability Implementation: Significant investment required for providers, particularly smaller practices, to upgrade systems and processes.

    - Governance and Trust Frameworks: Establishing clear rules and trusted agreements for data sharing (e.g., TEFCA's role).

PC–9. Given that the Blue Button 2.0 API only includes basic patient demographics, Medicare coverage, and claims data (Part A, B, D), what additional CMS data sources do developers view as most valuable for inclusion in the API to enable more useful digital products for patients and caretakers?

From Cohere Health's perspective, the most valuable additions would be:

- Detailed Clinical/Cost Data from Medicare FFS Encounters: Beyond claims, access to structured clinical data from FFS providers (diagnoses, procedures, lab results, medications, immunizations, vital signs). Additionally, cost data would be valuable, particularly to support price transparency and care planning.
- Medicare FFS Clinical Guidelines: The specific guidelines (NCDs/LCDs) applied to patient care for transparent medical necessity determination.
- Provider Quality and Performance Data: More granular data on specific provider outcomes, efficiency, and adherence to evidence-based practices (potentially drawing from PA data analysis to identify high-value providers).
- Patient-Reported Outcome Measures (PROMs): If CMS collects these, making them available in a usable format.

PC–10. How is the Trusted Exchange Framework and Common AgreementTM (TEFCATM) currently helping to advance patient access to health information in the real world?

- While TEFCA represents a critical step towards nationwide interoperability, its real-world impact on directly advancing patient access to health information is still nascent from Cohere Health's observation. The primary focus initially appears to be on provider-to-provider and payer-to-payer exchange for treatment, payment, and healthcare operations. The long-term promise is that as QHINs mature and data flows become more robust, patients will indirectly benefit from a more comprehensive health record

d. What standards are you aware of that are currently working well to advance access and existing exchange purposes?

- FHIR (Fast Healthcare Interoperability Resources): Particularly the US Core Implementation Guide

e. What standards are you aware of that are not currently in wide use, but could improve data access and integration?

- The following standards, which Cohere deploys in its system, are gaining traction across the market and have the opportunity to improve data access and integration:
    - FHIR Bulk Data API: For efficient, large-scale data export for population-level analytics and reporting, which could indirectly benefit patients through improved care management.
    - Clinical Decision Support: For embedding real-time, context-aware clinical decision support into workflows, which could improve care quality and potentially reduce inappropriate care that leads to prior authorization denials.
    - SMART on FHIR: For launching patient-facing applications from within EHRs, enabling seamless integration of digital health tools.

    - Standardized Prior Authorization APIs (e.g., Da Vinci CRD/DTR/PAS): While still gaining traction, these are crucial for automating and streamlining the prior authorization process from end to end, directly impacting patient access to care.
    - 270 Standards: The EDI 270 transaction set is another standardized format that can be more commonly leveraged to send inquiries about a patient's healthcare eligibility and benefit details.

f. Are there redundant standards, protocols, or channels that should be consolidated?**

- Yes, the proliferation of proprietary APIs and data formats used by individual EHR vendors and payers creates significant redundancy and burden. Consolidation around robust, open, and standardized FHIR APIs is essential.

g. Are there adequate alternatives outside of TEFCA for achieving widespread patient access to their health information?

- Currently, adequate alternatives for widespread patient access are lacking. Many patient portals are siloed. While individual patient-facing apps leveraging FHIR APIs are emerging, their ability to aggregate comprehensive data across all providers and payers without a unified framework like TEFCA remains limited. TEFCA, despite its early stages, is the most promising path to truly widespread data exchange.

PC–11. How are health information exchanges (HIEs) currently helping to advance patient access to health information in the real world?

- HIEs play a valuable role, particularly at the regional or state level, in aggregating and sharing patient data among participating providers. This often improves information availability for continuity of care during transitions and for emergency care. Some HIEs offer patient portals, or contribute data to larger patient-facing platforms, thereby improving patient access to their records.

a. How valuable, available, and accurate do you find the data they share to be?

- Valuable: Generally valuable for care coordination, especially for longitudinal views within a specific region or health system.
- Availability: Varies significantly by HIE. Some have broad participation, others are more limited.
- Accuracy: Generally considered accurate for the data they ingest, but completeness can be an issue if not all relevant providers participate.
- Limitations: The data may not always be real-time, comprehensive across all care settings (e.g., fragmented from different HIEs or non-participating entities).

b. What changes would you suggest?

- Increased Interoperability *between* HIEs: Ensure seamless data exchange across different HIEs, perhaps leveraging TEFCA.
- Standardization of Data Elements and Quality: Mandate adherence to USCDI and other standards to improve data consistency.
- Integration of Prior Authorization Data: Encourage HIEs to incorporate structured PA requests, decisions, and clinical criteria into their data exchange.
- Enhanced Patient-facing Capabilities: Support HIEs in developing more robust and user-friendly patient portals or APIs for patient access.

c. Are there particular examples of high-performing HIE models that you believe should be propagated across markets?**

- Models that demonstrate strong governance, high provider participation rates, robust data quality, and a clear return on investment (e.g., through reduced readmissions, improved population health outcomes) should be studied and propagated. HIEs that have successfully moved beyond basic data exchange to support advanced analytics and care management would also offer valuable lessons.

d. What is the ongoing role of HIEs amidst other entities facilitating data exchange and broader frameworks for data exchange (for example, vendor health information networks, TEFCA, private exchange networks, etc.)?

- HIEs will continue to play a crucial role as regional or state-level aggregators and facilitators of data exchange, especially for community providers and public health initiatives. In the context of TEFCA, many HIEs are likely to evolve into or become participants within QHINs, leveraging their existing infrastructure and trusted relationships. They will likely coexist and interconnect with vendor HINs and private exchange networks, forming a multi-layered ecosystem for data flow. Their role will be to ensure comprehensive local data coverage and support use cases specific to their communities, while linking into the broader national framework.

PC–12. What are the most valuable operational health data use cases for patients and caregivers that, if addressed, would create more efficient care navigation or eliminate barriers to competition among providers or both?

The most valuable operational health data use cases are those that address transparency and immediacy in the healthcare journey:

- Provider Navigation: Beyond basic directories, linking provider profiles to specific conditions and quality outcomes and ensuring provider directories are simplified and usable so patients can identify and select the best provider for their unique needs.
- Cost Estimates: Reliable and data-driven cost estimates are crucial for patients to make informed financial decisions about their care. The current price transparency efforts do not make this information readily available for patients navigating care.
- Viewing Provider Schedule Availability: Essential for efficient appointment booking and reducing delays in accessing care and using applications for appointment management to ensure convenience and patient care adherence.
- Real-time Prior Authorization Status: Knowing the exact status of a prior authorization request, clinical criteria, and expected decision. This is paramount for patient peace of mind and access to care and is an approach that Cohere champions in our platform today by offering transparency around the prior authorization process to physicians or patient care planning/discussions and delivering real-time clinical decisions, when appropriate.
- Proactive Care Path Nudges: Receiving automated, personalized recommendations for next steps in their care, based on evidence-based guidelines, as Cohere Health's intelligent prior authorization system provides to the ordering physician directly in the technology platform. This allows patients to feel informed and empowered about their care, and also presents an opportunity for shared clinical decision making conversations between patient and provider.
- Clinical Trial Studies: Finally, patients would also benefit from access to reliable data to navigate clinical trials. Today, the onus to provide information on clinical trials typically is in the hands of the physician, however, patients have a right to understand clinical trial sites and where they can explore more novel options for their care.

b. What use cases are possible today?

- Basic appointment scheduling (often within a single provider system).
- Accessing some lab results and basic demographics and clinical summaries via patient portals.
- Limited provider directories.
- Blue Button 2.0 provides claims history.

c. What should be possible in the near future?

- Real-time, Transparent Prior Authorization Status Accessible to Patients and Caregivers: This is a core capability Cohere Health is driving already in the market today as the leading technology company dedicated to evidence-based clinical decision making and real-time prior authorization.
- Automated, secure exchange of comprehensive clinical data across disparate providers for continuity of care.
- Machine-readable, actionable clinical guidelines and medical necessity criteria for prior auth available to patients and caregivers.
- Integrated appointment management across multiple providers.
- Binding cost estimates available prior to service.
- AI-powered care navigation that guides patients through their care journey, suggesting optimal providers and services based on appropriateness metrics derived from prior authorization data.

d. What would be very valuable but may be very hard to achieve?

- Truly unified longitudinal health record across all payers and providers globally: This requires overcoming immense technical, legal, and political hurdles.
- Predictive analytics that proactively identify and mitigate health risks at an individual level: Cohere Health's AI models can identify inappropriate care patterns, truly personalized, real-time risk mitigation requires vast, integrated data and advanced ethical AI frameworks. Other technology companies may offer similar approaches, likely driven by claims data, however Cohere's rich prior authorization data assets offer real-time insights.
- Automated, seamless integration of all SDOH data into individual care plans: Requires significant data collection, standardization, and integration efforts across health and social services.

C. Providers

Digital Health Apps

PR–1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved digital health products for their patients?

CMS and its partners can encourage provider adoption by:

- Demonstrate Value & ROI: Clearly articulate the tangible benefits for providers, such as reductions in administrative burden and offer reimbursement for specific digital health services that demonstrably improve care or reduce costs. Link digital health adoption to value-based care models and quality performance measures.
- Streamlined Integration and Interoperability: Prioritize digital health solutions that offer seamless integration with EHRs and clinical workflows, or have the opportunity to connect to existing channels like the fax machine. Interoperability is key to ensuring advancements in technology can modernize processes like prior authorization, however, today, most healthcare organizations, especially smaller individual physicians in both urban and rural areas, are still using fax machines. In fact, about 50 percent of all prior authorization submissions are non-electric (e.g.,

fax or phone). Fax conversion, which Cohere deploys, is one strategy that would ensure that, regardless of the means of submission (e.g., phone, fax, portal), there is no “wrong door” for submission.

- Education and Training: Provide comprehensive training and ongoing support for providers and their staff on how to effectively use digital health tools.
- Address Interoperability Gaps: Aggressively enforce information blocking policies and mandate standardized APIs (especially for Prior Authorization) to ensure that digital health products can truly exchange data. Today, the legacy portal systems lack simple design and interactiveness to connect with smaller systems, which is critical for adoption and true interoperability.
- Rural Support: Offer grants for rural providers to invest in necessary broadband infrastructure and digital health technology, and provide technical assistance tailored to their unique needs.

a.What are the current obstacles?

- Administrative Burden: Perceived or actual additional work required to integrate and use new digital tools.
- Lack of Interoperability: Siloed EHR systems and proprietary data formats prevent seamless data flow.
- Cost of Adoption: Financial investment in new technologies, training, and infrastructure.
- Lack of Clear ROI: Difficulty in demonstrating a direct financial or clinical benefit that justifies the investment.
- Digital Divide: Limited broadband access and technological expertise/investment in rural areas.
- Information Blocking: Practices by some legacy vendors that hinder access to or exchange of EHI.
- Trust and Efficacy Concerns: Skepticism regarding the effectiveness or security of unproven digital health products.
- Provider Burnout: Limited capacity or willingness to adopt new tools amidst existing pressures.

b. What information should providers share with patients when using digital products in the provision of their care?

- This largely depends on the nature of the digital product (e.g., if it is a supporting a back-end administrative or clinical decision-making function versus a patient-facing digital health or care management app the approaches would naturally differ), however, in general for applications patients are directly engaged with:
- Purpose and Benefits: Clearly explain how the technology will be used, what problem it solves, and how it will benefit the patient's care.
- Data Privacy and Security: Assure patients about how their health information will be protected and used.
- Roles and Responsibilities: Define what the patient is expected to do, what the app will do, and what the provider's role will be.
- Support and Troubleshooting: Provide clear instructions on whom to contact for technical issues or clinical questions related to the app.
- Emergency Protocols: Crucially, clarify that the app is not for emergencies and provide instructions for urgent situations.

c. What responsibilities do providers have when recommending use of a digital product by a patient?

- Due Diligence: Evaluate the efficacy, safety, and security of the digital product before recommending it.
- Informed Consent: Ensure patients understand the implications of using the product, including data sharing.
- Integration with Workflow: Ensure the product can be effectively integrated into their clinical workflow to act on the data it generates.
- Monitoring and Follow-up: Establish a plan for monitoring patient engagement and outcomes related to the digital product.
- Patient Education: Provide clear, accessible instructions and ongoing support for patients.
- Ethical Use of AI: For AI-driven products, ensure understanding of how the AI functions and its limitations, which is key and core in Cohere's AI practices.

PR–2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated?

- Obstacles:
    - Lack of Data Standardization: Inconsistent data formats and requirements make it difficult to develop applications that work across multiple EHRs.
    - Proprietary APIs and Information Blocking: EHR vendors often use proprietary APIs or engage in information blocking, hindering third-party developers from accessing and utilizing clinical data. This forces developers to build point-to-point integrations, increasing cost and complexity.
    - Fragmented Clinical Guidelines: Today, there is a duopoly in the clinical guideline industry and providers face inconsistencies and confusion when submitting prior authorizations and making key clinical decisions to render care.
    - Regulatory Complexity: A patchwork of state and federal regulations around data sharing, privacy, and billing.
    - High Cost of Integration: Developing and maintaining integrations with numerous EHR systems is expensive.
    - Usability Issues with Existing EHRs: Clunky interfaces and workflows in legacy EHRs make it difficult to integrate new, innovative applications seamlessly.
    - Limited Access to Real-World Data: Difficulty in obtaining de-identified real-world data for training and validating AI/ML models.
    - Unclear standards for AI governance: Creates confusion and mis-trust on the appropriate use of AI within healthcare. Further, the patchwork nature of state level regulations will greatly increase compliance burdens and limit the benefits of AI usage.
- Mitigation:
    - Strict Enforcement of Information Blocking Rules: CMS and ONC must vigorously penalize information blocking practices to ensure true interoperability.
    - Mandate Standardized FHIR APIs: Require all certified EHRs to support a comprehensive set of FHIR APIs, including those for Prior Authorization (e.g., Da Vinci CRD/DTR/PAS), Patient Access, Provider Access, and Bulk FHIR.
    - National Guideline Clearinghouse: As Cohere previously suggested, CMS could issue an RFP for a nationwide clearinghouse to standardize clinical guidelines in an open-source, public-good framework. This would make guideline-based automation more feasible.

    - Incentivize EHR Modernization: Provide incentives for EHR vendors to adopt more open architectures and developer-friendly platforms.
    - Cloud-Based Interoperability Platforms: Support the development of secure, cloud-based platforms that can facilitate data exchange and integration.
    - Reduce Redundant Reporting: Consolidate quality reporting requirements and leverage bulk FHIR exports to reduce the burden on providers.

PR–3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? Please address all of the following:

- It is critically important for healthcare delivery and interoperability that all data in an EHR system be accessible for exchange, regardless of format. The current reality, where approximately 50% of PA submissions are non-electronic, highlights this urgent need. This means that, despite best efforts, interoperability with EMRs will not unlock all of the potential and it is critical that the healthcare system finds new ways to migrate fax to simpler apps and portals. Across Cohere's book of business, we have nearly 90 percent portal adoption which is significantly higher than national average, however, we find that it is critical to still support the 10 percent that don't submit via electronic means.

a. Current challenges in accessing different data formats.

- Scanned Documents/Faxes: These are essentially images, requiring manual review or complex OCR/AI processing to extract usable data. This is time-consuming, prone to error, and a major bottleneck for prior authorization. Fax conversion, which Cohere deploys, is one strategy that would ensure that, regardless of the means of submission (e.g., phone, fax, portal), there is no "wrong door" for submission.
- Free Text Notes:While rich in clinical detail, extracting structured, actionable information from free text requires advanced Natural Language Processing (NLP) and AI, which many systems lack.
- Lack of Standardization: Even structured data fields can vary in terminology, coding, and granularity across different EHRs, requiring significant mapping and normalization.
- Proprietary Data Models: Data stored in proprietary formats within EHRs is difficult to access and translate without vendor-specific tools.

b. Impact on patient care quality.

- Care Delays:Inability to quickly access complete patient information, particularly from faxes and scanned documents, directly leads to delays in diagnosis and treatment, as seen in the PA process. Cohere's intelligent PA has reduced care delays by 80%.
- Medical Errors:Incomplete or fragmented patient data can lead to medication errors, redundant tests, and inappropriate treatment decisions.
- Provider Burnout: Clinicians spend excessive time sifting through unorganized or inaccessible data, contributing to frustration and burnout.
- Fragmented Care: Lack of a holistic patient view hinders effective care coordination across different specialties and care settings.

c. Technical barriers to full data accessibility.

- Legacy Systems: Many legacy EHRs were not designed with interoperability in mind and lack modern API capabilities and also often strategically block integration with smaller, newer technologies as previously mentioned.

- Cost of Data Transformation: The expense and technical complexity of converting unstructured or proprietary data into standardized, exchangeable formats.
- AI/NLP Maturity: While rapidly advancing, AI/NLP for accurate extraction from complex clinical free text still requires significant development and validation.
- Lack of Common Language: No common "language" across all healthcare data systems for consistent interpretation.
- Fragmented Clinical Guidelines: As previously mentioned, today there is a duopoly in the clinical guideline industry and providers face inconsistencies and confusion when submitting prior authorizations and making key clinical decisions to render care.

d. Cost or privacy implications of making all data formats interoperable.

- Cost: Initial investment would be significant for EHR vendors and providers to upgrade systems, implement advanced NLP, and standardize data. However, the long-term savings from reduced administrative waste, improved patient outcomes, and eliminated redundant tests would likely far outweigh these costs.
- Privacy: This is a paramount concern. Making all data interoperable requires robust technical safeguards (e.g., encryption, access controls) and strong legal frameworks (e.g., HIPAA, TEFCA) to ensure data is only accessed by authorized individuals for permissible purposes. Responsible AI development, which is at the forefront of Cohere's AI approach, including bias identification and mitigation, is also critical to ensure privacy and equity are upheld when processing sensitive data.

e. Priority level compared to other interoperability needs.

- Full data accessibility, regardless of format, is a **top-tier priority** and foundational to achieving all other interoperability needs. Without it, even the most robust APIs or networks will only exchange a subset of the patient's crucial information, limiting their true impact. It is a necessary prerequisite for advanced use cases like AI-driven clinical decision support and comprehensive population health analytics.

PR–4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment?

- Standardized FHIR Administrative APIs: Develop and mandate FHIR APIs for administrative workflows.
- Enforcement of API Accessibility: Policies must ensure that EHR vendors and other health IT developers provide robust, developer-friendly access to these administrative APIs without excessive cost or proprietary hurdles.
- Clear Consent Mechanisms: Policies need to define clear, granular patient consent mechanisms for third-party apps to access and utilize their administrative data.
- Certification Requirements: ONC Health IT Certification Program should include criteria for supporting these specific administrative workflow APIs.

2. Data Exchange

PR–5. Which of the following FHIR APIs and capabilities do you already support or utilize in your provider organization's systems, directly or through an intermediary? For each, describe the transaction model, use case, whether you use individual queries or bulk transactions, and any constraints:

As Cohere Health, we primarily operate as a technology platform and intermediary, not a direct "provider organization" in the traditional sense of delivering care. However, we integrate with and enable these functionalities for our payer and provider partners. Our focus is on the efficient exchange of healthcare information to facilitate prior authorization. Please see below overview of how we support or utilize the specified FHIR APIs and capabilities:

a. Patient Access API:

- Transaction Model: While Cohere Health doesn't directly expose a Patient Access API to patients, we facilitate data exchange that can support a payer's or provider's Patient Access API requirements. This typically involves individual queries for specific patient data.
- Prior Authorization Status: Cohere Health's portal also features the capability to provide prior authorization status, offering transparency to patients and their providers so they can understand the timeline for their prior authorization request review.
- Use Case: Our platform processes and synthesizes patient clinical data (e.g., demographics, conditions, procedures) received from provider EHRs to support prior authorization requests. This data, once authorized, can then be surfaced by the payer or provider to their patient via their respective Patient Access APIs. We ensure the data we handle is structured in a FHIR-compliant manner to aid this.
- Individual Queries or Bulk Transactions: Predominantly individual queries for patient-specific clinical information relevant to a prior authorization request. We receive data for a single patient's authorization.

b. Standardized API for Patient and Population Services:

- Transaction Model: We support the underlying data exchange that contributes to this API, particularly in the context of our prior authorization workflows. This often involves individual queries for patient-specific data, but our aggregated data can also support bulk transactions for population health analytics on the payer side, once de-identified or appropriately consented.
- Use Case: Within the prior authorization process, we consume and normalize clinical data from various provider systems. This data, when aggregated and analyzed by our payer partners, can inform population health initiatives and care management strategies. For example, identifying patterns in approved vs. denied authorizations for specific conditions.
- Individual Queries or Bulk Transactions: Primarily individual queries for clinical data related to a specific prior authorization. We have the capability to support bulk transactions for aggregated, de-identified data for population-level insights for our payer partners, but this is handled by the payer's analytics systems, utilizing data we've processed.

c. Provider Directory API:

- Transaction Model: Cohere Health utilizes Provider Directory information to route prior authorization requests appropriately and to ensure that authorizations are linked to the correct providers. This involves individual queries to ascertain provider details.
- Use Case: We integrate with and consume data from provider directories (either directly from health systems or through payer-maintained directories) to validate provider credentials, specialties, and affiliations during the prior authorization submission and review process. This ensures accurate communication and routing of requests.
- Individual Queries or Bulk Transactions: Predominantly individual queries to retrieve information for specific providers involved in a prior authorization request.

d. Provider Access API:

- Transaction Model: Cohere Health acts as a platform that provides a "Provider Access" experience, facilitating the submission and tracking of prior authorizations. This is not a direct FHIR Provider Access API in the sense of a provider directly querying a payer's internal systems, but rather our platform serves as the API endpoint for providers to interact with the prior authorization process. It involves a combination of individual queries and resource submissions.
- Use Case: Our primary offering is a streamlined digital experience for providers to submit prior authorization requests, receive status updates, and view determination letters. This involves FHIR-based exchange of administrative and clinical data related to the authorization.
- Individual Queries or Bulk Transactions: Primarily individual queries and resource submissions for specific prior authorization requests. Providers submit individual authorization requests and query the status of those specific requests.

e. Payer-to-Payer API:

- Transaction Model: Our platform is designed to facilitate the data exchange that would underpin a Payer-to-Payer API. This is crucial for continuity of care and avoiding redundant prior authorizations when a patient switches payers. It would likely involve bulk transactions for historical authorization data.
- Use Case: In a scenario where a patient changes payers, our platform's ability to ingest and structure prior authorization data in FHIR format could be instrumental in transferring relevant historical authorizations between the old and new payers. This capability is in development and aligns with the evolving regulatory landscape.
- Individual Queries or Bulk Transactions: We anticipate that future Payer-to-Payer API interactions would predominantly involve bulk transactions for transferring a patient's historical prior authorization data.

f. Prior Authorization API (e.g., Da Vinci CRD, DTR, PAS):

- Transaction Model: This is a core competency and primary focus for Cohere Health. We actively support and are building upon the Da Vinci Use Cases (Coverage Requirements Discovery (CRD), Documentation Templates and Rules (DTR), and Prior Authorization Support (PAS)). This involves individual queries and resource submissions for real-time interactions.
- Use Case: Our platform directly implements the functionalities described in the Da Vinci PAS IG. This includes:
    - CRD: Responding to coverage requirements queries from provider EHRs, providing real-time information on whether a prior authorization is needed for a specific service.
    - DTR: Guiding providers through the necessary documentation and data elements required for a prior authorization submission.
    - PAS: Enabling the submission of a complete prior authorization request (including clinical documentation) and receiving status updates and determinations.
- Individual Queries or Bulk Transactions: Primarily individual queries and resource submissions for each prior authorization request. The nature of prior authorization is inherently transaction-based for individual patient services.
- Constraints:
    - The maturity and adoption of these standards across the industry. The variability in payer business rules and clinical policies, which need to be translated into machine-readable rules. Interoperability challenges with diverse EHR vendor implementations.

    - Additionally, the regulations and implementation guides (IGs) do not account for information the auth submitters of additional action items (e.g., the service may be better suited to occur in an outpatient setting instead of inpatient).
    - The regulations and IGs require the administrative staff at the provider to answer questions. It limits vendors that are able to extract data from primary source clinical notes to meet the requirements of the authorization and reintroduces burden to providers who stopped having to answer these questions.

g. Bulk FHIR—Do you support Group ID-based access filtering for population-specific queries?

- Transaction Model: Yes, Cohere Health has the capability to support bulk FHIR data extraction and can incorporate Group ID-based access filtering. While our primary transactional focus is on individual prior authorizations, our underlying data architecture can support population-level data analysis.
- Use Case: For our payer partners, we can provide de-identified or aggregated data extracts based on specific population groups. This supports population health management, quality reporting, and actuarial analysis. This is typically done through secure data extracts rather than real-time API queries for bulk data, but the underlying data can be organized for such purposes.
- Individual Queries or Bulk Transactions: This is specifically geared towards bulk transactions for population-specific queries.
- Constraints: Strict adherence to data de-identification standards and privacy regulations (HIPAA). The definition and consistency of "Group IDs" across different payer systems. The technical overhead of generating and transferring large datasets securely.

Cohere Health is deeply embedded in the FHIR ecosystem, primarily as an enabler and facilitator of compliant and efficient prior authorization workflows. We leverage and integrate with various FHIR APIs to reduce administrative burden, accelerate care, and improve health outcomes for all stakeholders.

PR–7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? How can the burden of increasing data availability and sharing be mitigated for providers? Are there ways that workflows or metrics that providers are already motivated to optimize for that could be reused for, or combined with, efforts needed to support interoperability?

- Strategies to Support Data Availability:
    - Aggressive Enforcement of Information Blocking: This is paramount. Without it, technical advancements will be undermined.
    - Mandate Standardized FHIR APIs for all data types: Not just claims or basic clinical data, but also granular details like prior authorization status, appeals, and comprehensive clinical notes.
    - Incentivize EHR Upgrades: Encourage providers to adopt modern, API-enabled EHRs through financial incentives to ensure there is no wrong door for request submission.
    - Provide Technical Assistance: Offer resources, training, and support for providers, especially small and rural practices, to implement interoperable systems.
    - Promote AI for Data Extraction and Structuring: Encourage and support the development and adoption of AI tools that can extract structured, high-quality data from unstructured formats (e.g., free text notes, scanned documents) within the EHR. This would transform unusable data into interoperable data.
- Mitigating Burden for Providers:

    - Reduce Redundant Reporting: Consolidate quality reporting requirements by allowing direct, automated data extraction via APIs from EHRs to CMS/payers, rather than requiring manual abstraction or multiple submissions.
    - "EZ Pass" for High Performers: As Cohere Health has suggested, leverage prior authorization data to identify high-performing providers who consistently adhere to evidence-based guidelines and provide high-value care. These providers could then be granted streamlined prior authorization processes or reduced reporting requirements.
    - Automate Prior Authorization: This is Cohere Health's core mission. By reducing the time providers spend on authorizations by nearly 40%, we significantly mitigate administrative burden while also improving care.
    - Embed Interoperability into Clinical Workflows: Design policies and tools that make data sharing a natural, effortless part of daily clinical practice, rather than an add-on task.
    - Incentivize Reciprocal Access: Reward providers who actively share their data by granting them easier access to data from other entities.

- Reusing / Combining Workflows and Metrics:
    - Quality Measurement: The data needed for quality measurement (e.g., patient outcomes, adherence to guidelines) can be acquired through interoperability. By using Bulk FHIR exports for quality reporting, CMS can reduce provider burden while simultaneously gaining richer data for analytics.
    - Value-Based Care (VBC) Reporting: VBC models already require data on appropriateness of care and outcomes. Prior authorization data, in particular, can be leveraged to develop metrics for appropriateness of care within total cost of care models, allowing CMS and payers to monitor and reward physicians based on practice patterns without imposing additional reporting.
    - Clinical Decision Support: Data used for CDS hooks (e.g., patient problem lists, medications) is also essential for interoperability. Integrating CDS capabilities that guide providers to appropriate care, thereby reducing inappropriate use (as Cohere's system does by flagging contraindications), directly improves the quality of data being shared.

- AI Governance Given the proliferation of new AI solutions within the market, it is imperative to acknowledge that not all AI is responsible AI. Cohere Health adheres to robust principles in the development of our responsible AI capabilities and advocates for their widespread adoption within the healthcare delivery system and encourages CMS to keep comparable principles and policies at the forefront. Cohere’s approach to AI encompasses:
    - Modeling Approaches: Rigorous development and testing, conducted with the involvement of in-house clinical experts, to automate medical necessity evaluation utilizing comprehensive longitudinal patient profiles, which leverage both deterministic and predictive models.
    - Business Evaluation: Employment of a proprietary evaluation harness to rigorously test AI models against meticulously hand-labeled "golden" datasets, thereby ensuring that quality and safety standards are consistently met prior to deployment.
    - Bias Identification & Mitigation: Execution of regular manual audits and deployment of a robust monitoring harness that surveils the entire authorization "population," automatically flagging irregularities pertaining to subpopulations of interest, high-risk cohorts, or protected classes for immediate investigation by data scientists, clinical experts, and compliance teams.
    - Deployment: Requirement of rule simulation during testing before any model deployment to ensure forecasted behavior and impact are thoroughly understood. Predictive models

are closely monitored for efficiency and quality in decision-making, supported by bi-weekly code reviews and ongoing change management.
    - Performance Monitoring: High-volume, low-latency ML systems necessitate real-time tracking to ensure the reliability and scalability of model inferences. Our monitoring harness regularly scans every model for data availability and drift, as well as indicators of bias or out-of-class/out-of-distribution data, routing irregular performance for immediate investigation.

PR–8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers?

a. What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?

- Benefits:
    - Significant Burden Reduction: Eliminates manual data abstraction, separate reporting tools, and redundant submissions.
    - Improved Data Quality: Reduces human error associated with manual processes.
    - Timely and Comprehensive Data: Provide richer, more frequent data for real-time analysis and program improvement.
    - Interoperability Investment Consolidation: Providers' investments in FHIR APIs for other purposes (e.g., patient access) can be dually purposed for quality reporting.
    - CMS-Calculated Metrics: CMS can reduce the burden by performing quality metrics calculations leveraging Bulk FHIR data exports. This shifts the calculation burden from individual providers to CMS.
- Downsides:
    - EHR Vendor Readiness: Requires all EHRs to fully support Bulk FHIR exports and ensure data consistency.
    - Data Volume and Storage: CMS would need robust infrastructure to handle and process large volumes of data.
    - Data Normalization and Quality: Ensuring consistency and accuracy of data received from various EHRs.
    - Privacy Concerns: Ensuring appropriate de-identification and security for population-level data.

b. In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?

- Standardized FHIR API Mandate: Require FHIR APIs for *all* data exchange, including quality reporting.
- Centralized Reporting Platforms: CMS could establish a secure, centralized platform where providers submit data once, and it is then used for various reporting requirements (quality, VBC, public health).
- "Report Once" Philosophy: Design policies that encourage health IT systems to generate data once, in a standardized format, which can then be automatically submitted for multiple reporting purposes.
- Leverage Prior Authorization Data for Quality: Prior authorization data itself contains rich insights into care appropriateness and can be used to track quality and identify patterns of inappropriate

care. Cohere Health's data can help CMS optimize which codes should be on a PA list by identifying where inappropriate use most frequently occurs.

4. Information Blocking

PR–12. Should ASTP/ONC consider removing or revising any of the information blocking exceptions or conditions within the exceptions (45 CFR part 171, subparts B through D) to further the access, exchange, and use of electronic health information (EHI) and to promote market competition?

Yes. From Cohere Health's experience, particularly in the context of prior authorization and the challenges of accessing complete clinical data, we believe ASTP/ONC should continuously review and consider revising or refining information blocking exceptions, with a strong bias toward promoting access, exchange, and use of EHI. The goal should be to remove any exceptions that are being exploited to hinder legitimate data sharing for treatment, payment, and healthcare operations. In addition to patient care challenges posed by information-blocking practices of legacy technology companies and EMRs/EHRs, it also thwarts innovation in the healthcare delivery system, particularly among technology startup organizations which is a detriment to the economy.

Specific areas for review could include:

- Content and Manner Exception: Ensure that this exception is not used to limit the types of EHI made available (e.g., intentionally excluding detailed imaging reports or prior authorization decisions) or to impose overly burdensome or non-standard "manner" requirements that effectively block access.
- Fees Exception: Carefully scrutinize whether fees for EHI access are genuinely cost-recovery or are designed to be prohibitive barriers.
- Security and Privacy Exceptions: While essential, ensure these are not overly broadly interpreted to justify blocking data that could be securely shared.
- Interference Exception: Ensure that legitimate competition or innovation is not being labeled as "interference" to justify blocking.

Revising exceptions should focus on clarity, specificity, and preventing loopholes that allow actors to avoid their obligations. The ultimate impact should be to promote a more competitive market for digital health and technology companies by lowering the barrier to data access for innovators.

D. Payers

PA–2. How can CMS encourage payers to accelerate the implementation and utilization of APIs for patients, providers, and other payers, similar to the Blue Button 2.0 and Data at the Point of Care APIs released by CMS?

- Mandate API Implementation: Implement regulations requiring all payers to expose standardized FHIR APIs for patient, provider, and payer-to-payer data exchange, particularly focusing on the Prior Authorization API (Da Vinci Burden Reduction APIs).
- Performance-Based Incentives: Offer financial incentives to organizations that demonstrate early and robust adoption and utilization of these APIs, with measurable improvements in data exchange and patient access.
- Public Reporting of API Compliance: Publish data on payer adherence to API mandates, creating a competitive pressure to comply and excel.

- Technical Assistance and Best Practices: Provide comprehensive technical guidance, reference implementations, and share best practices from early adopters.
- Integrate API Usage into Quality Programs: Incorporate metrics related to API utilization and data exchange into existing quality measurement programs.
- Leverage AI for API Performance Monitoring: Use AI to monitor API performance, identify bottlenecks, and ensure data quality, holding payers accountable.

PA–4. What would be the value to payers of a nationwide provider directory that included FHIR end points and used digital identity credentials?

A nationwide provider directory with FHIR endpoints and digital identity credentials would be immensely valuable to payers by:

- Streamlined Referrals and Network Management: Easier identification of in-network providers, specialists, and care facilities, facilitating seamless patient referrals and network adequacy assessments.
- Efficient Prior Authorization Routing: Automated routing of prior authorization requests directly to the correct provider's FHIR endpoint, reducing faxes and manual lookups.
- Improved Communication and Coordination: Secure and direct communication channels with providers via their verified FHIR endpoints, improving care coordination.
- Enhanced Data Exchange: Knowing a provider's FHIR endpoint and verified identity enables direct and secure exchange of clinical and administrative data for treatment, payment, and operations.
- Reduced Administrative Costs: Automating provider discovery and communication would significantly cut down on administrative overhead.
- Fraud, Waste, and Abuse Detection: Verifying provider identities and linking them to endpoints can help in identifying suspicious patterns.

PA–5. What are ways payers can help with simplifying clinical quality data responsibilities of providers?

a. How interested are payers and providers in EHR technology advances that enable bulk extraction of clinical quality data from EHRs to payers to allow them to do the calculations instead of the provider-side technology?

- Payers: Highly interested. This would provide them with more comprehensive and timely data for quality reporting, population health management, and value-based care performance assessment. It would also reduce their need to rely on provider-side data abstraction or attestation.
- Providers: Very interested. This would significantly reduce their administrative burden associated with quality reporting, freeing up staff time for patient care. It aligns with the "report once" philosophy.
- Cohere Health's View: We believe this is a critical step towards simplifying quality reporting. Our expertise in extracting and analyzing clinical data (e.g., identifying inappropriate care patterns) positions us to understand the immense value of this approach.

b. In what ways can the interoperability and quality reporting responsibilities of providers to both CMS and other payers be consolidated so investments can be dually purposed? Are there technologies payers might leverage that would support access to real time quality data for healthcare providers to inform clinical care in addition to simplifying reporting processes?

- Consolidation:
    - Standardized Data Formats (FHIR, USCDI): Require all quality data submissions (to CMS, payers, registries) to use the same standardized FHIR-based format, built on USCDI.
    - Centralized Data Repositories: Encourage or mandate that providers submit their quality data to a central, secure repository (e.g., a TEFCA-enabled QHIN) from which both CMS and payers can access it, rather than individual submissions to each entity.
    - Payer-Calculated Metrics: Require payers to calculate quality metrics from the raw, standardized data submitted by providers, rather than providers having to pre-calculate and submit.
- Technologies Payers Can Leverage for Real-Time Quality Data (to inform clinical care):
    - AI/ML for Real-Time Insights: Payers can use AI/ML to analyze incoming clinical data (e.g., from prior authorization requests, claims data) in real-time to identify potential care gaps, adherence to guidelines, or opportunities for intervention. For example, Cohere Health's platform uses AI/ML models to identify clinical appropriateness before care.
    - CDS Hooks and SMART on FHIR: Payers could develop or fund digital health tools that integrate into provider EHRs via these standards, providing real-time feedback on quality metrics, care gaps, or appropriate care pathways.
    - Data Dashboards with Benchmarking: Provide providers with secure access to dashboards showing their own quality performance compared to peers and benchmarks, updated in near real-time.
    - Proactive Nudges: Leverage insights from their data analytics to send proactive "nudges" to providers (e.g., "Patient X is due for Y screening") based on established quality measures.

E. Technology Vendors, Data Providers, and Networks

1. Ecosystem

TD–1. What short term (in the next 2 years) and longer-term steps can CMS take to stimulate developer interest in building digital health products for Medicare beneficiaries and caregivers?

- Short Term (next 2 years):
    - Clear and Consistent API Mandates: Provide unequivocal direction on requiring robust FHIR APIs for all relevant data types, including prior authorization status and clinical criteria, from EHRs and payers.
    - Standardized Clinical Guidelines Clearinghouse: Establish a central, open-source repository for clinical guidelines, specifically around medical necessity criteria, as previously suggested by Cohere. This reduces ambiguity for developers building decision support tools.
    - Enforce Information Blocking: Aggressively penalize information blocking to create a truly level playing field for data access. This is fundamental.
    - Publish Robust Test Data and Environments: Offer high-quality, de-identified Medicare claims and clinical data in a standardized (FHIR) format for developers to test and build applications.
    - Private Sector Collaboration: Sponsor innovation challenges and pilot opportunities within Medicare FFS focused on specific Medicare beneficiary needs, with prizes for effective digital health solutions, particularly those focused on combatting fraud, waste, and abuse. For example, Cohere would be thrilled to help CMS pilot its first ever AI-driven PA

solution within the Medicare FFS program, leveraging innovation afforded by the private sector. Adopting an approach for Medicare FFS that more closely resembles the approach in MA would be an effective way for CMS to capture savings to the program, identify inappropriateness and abuse patterns across the delivery system, and ensure patient safety and appropriate clinical care.

- Longer Term:
    - Responsible AI: Establish standards on key facts, model performance metrics, and safety measures every AI solution should be made available.
    - Value-Based Care Alignment: Structure VBC models to explicitly reward the use of digital health products that demonstrably improve outcomes and reduce costs.
    - National Digital Identity Framework: Champion a robust and ubiquitous digital identity system for patients and providers.
    - Sustainable Reimbursement Models: Develop clear and consistent reimbursement pathways for digital health services.

TD–2. Regarding CMS Data, to stimulate developer interest— a. What additional data would be most valuable if made available through CMS APIs?

From Cohere Health's perspective, as previously mentioned, the most valuable additions would be:detailed clinical data

- Detailed Clinical/Cost Data from Medicare FFS Encounters: Beyond claims, access to structured clinical data from FFS providers (diagnoses, procedures, lab results, medications, immunizations, vital signs). Additionally, cost data would be valuable, particularly to support price transparency and care planning.
- Medicare FFS Clinical Guidelines: The specific guidelines (NCDs/LCDs) applied to patient care for transparent medical necessity determination.
- Provider Quality and Performance Data: More granular data on specific provider outcomes, efficiency, and adherence to evidence-based practices (potentially drawing from PA data analysis to identify high-value providers).
- Patient-Reported Outcome Measures (PROMs): If CMS collects these, making them available.

b. What data sources are most valuable alongside the data available through the Blue Button 2.0 API?

c. What obstacles prevent accessing these data sources today?

d. What other APIs should CMS and ASTP/ONC consider including in program policies to unleash innovation and support patients and providers?

- Prior Authorization APIs (Da Vinci Burden Reduction APIs: CRD/DTR/PAS): Mandating these across all payers and providers is foundational for streamlining PA.
- Provider Scheduling APIs: For direct appointment booking and viewing availability.
- Patient Intake APIs: For auto-populating demographic and insurance information.
- Referral Management APIs: For tracking and managing referrals.
- CDS Hooks: For embedding real-time clinical decision support directly into EHR workflows.
- Bulk FHIR APIs: For efficient population-level data access for quality reporting and analytics.

3. Technical Standards and Certification

TD–4. How can CMS better encourage use of open, standards-based, publicly available APIs over proprietary APIs?

- Mandate FHIR Adoption: Require all certified health IT (EHRs, payer systems, digital health products) to implement and publicly attest to supporting a comprehensive set of FHIR APIs for data access and exchange.
- Information Blocking Enforcement: Rigorously enforce information blocking rules to explicitly prohibit practices that favor proprietary APIs or make it difficult to use open APIs.
- Certification Requirements: Strengthen ONC Health IT Certification Program criteria to prioritize API-enabled capabilities and compliance with open standards, making it harder for proprietary systems to achieve certification without robust open API support.
- Incentivize Open API Development: Offer financial incentives or grant programs for developers who build solutions using open, standards-based APIs.
- Provide Technical Resources: Develop and disseminate robust implementation guides, sample code, and testing tools for open APIs.
- Public Reporting: Publish information on health IT vendors' adherence to open API standards, creating transparency and competitive pressure.

TD–5. How could a nationwide provider directory of FHIR endpoints improve access to health information for patients, providers, and payers? Who should publish such a directory, and should users bear a cost? A nationwide provider directory of FHIR endpoints would be a game-changer for interoperability.

- Improved Access to EHI:
    - For Patients: Easier to find providers, understand their capabilities, and securely share their data with new care teams.
    - For Providers: Seamlessly locate and communicate with other providers for referrals, care coordination, and data exchange.
    - For Payers: Dramatically streamline processes like prior authorization routing, network management, and claims adjudication.

Who should publish:

- CMS/ONC: Given its national scope and importance as public infrastructure, a federal entity (CMS or ONC) is uniquely positioned to sponsor, oversee, or directly publish such a directory. This would ensure neutrality, broad adoption, and public trust.
- Alternatively, a Public-Private Partnership: A designated non-profit or public utility entity, governed by federal oversight, could manage it.

Should users bear a cost?

- Ideally, users (patients, providers, payers) should not bear a direct cost for accessing or querying the directory. It should be considered essential public good infrastructure for healthcare interoperability. The costs of development and maintenance should be covered through federal funding or perhaps a nominal, shared fee from health IT vendors or payers who benefit significantly from its existence.

TD–8. What are the most effective certification criteria and standards under the ONC Health IT Certification Program? The most effective criteria are those that directly support interoperability, patient access, and burden reduction.

- FHIR API-enabled capabilities: Criteria that mandate robust implementation of FHIR APIs for patient access, data exchange, and administrative workflows (including Prior Authorization APIs).
- Information Blocking Compliance: Strong criteria that ensure vendors actively facilitate, rather than hinder, data exchange. We believe these efforts can and should go even further to promote innovation and information-sharing.
- USCDI Adoption: Requiring adherence to USCDI for exchanged data.
- Security and Privacy: Criteria that ensure robust data protection.
- Attestation of Interoperability Performance: Going beyond simply having a feature, requiring attestation of actual performance in data exchange.
- Support for Digital Identity: Criteria that ensure systems can integrate with standard digital identity credentials.

TD–9. Regarding certification of health IT: a. What are the benefits of redefining certification to prioritize API-enabled capabilities over software functionality?

- Promotes Innovation: Fosters a vibrant ecosystem of third-party digital health applications by making data readily accessible.
- Increases Competition: Lowers barriers to entry for new developers, as they can build on a common data layer rather than needing deep integrations into each proprietary EHR.
- Enhances Interoperability: Moves beyond siloed functionality to truly enable data flow between disparate systems.
- Future-Proofing: APIs are more adaptable to evolving healthcare needs and new technologies (like AI).
- Reduces Vendor Lock-in: Patients and providers have more flexibility in choosing best-of-breed applications.

b. What would be the drawbacks?

- Implementation Burden: Requires significant investment from legacy EHR vendors to re-architect their systems for API-first design.
- Transition Period Challenges: Potential for disruption during the shift from functionality-focused to API-focused certification.
- API Security Concerns: Increased focus on securing APIs, as they expose data directly.
- Ensuring Usable APIs: Certification needs to ensure not just the presence of an API, but that it's well-documented, reliable, and performs as expected.

c. How could ASTP/ONC revise health IT certification criteria to require APIs to consistently support exchanging data from all aspects of the patient’s chart (for example, faxed records, free text, discrete data)?

- AI/NLP Capabilities: Require certified EHRs to include or integrate with AI/NLP capabilities that can extract structured data from unstructured formats (e.g., free text notes, scanned documents) and make that structured data available via APIs. This would apply to critical data needed for prior authorization and clinical decision-making.

- Define USCDI Extensions for Unstructured Data: Expand USCDI to include guidance on how to represent or reference information typically found in unstructured formats.
- Performance-Based Certification: Instead of just checking for an "API," require testing that demonstrates the API's ability to consistently exchange data from various parts of the chart with high fidelity and completeness.
- Incentivize Data Standardization at the Source: Reward vendors who proactively implement stricter data entry standards that result in more structured data.
- No Wrong Door: Reward vendors that adhere to a “no wrong door” approach of data submission and ingestion and can take information from fax, phone, free text, etc.

e. How could EHRs capable of bulk data transfer be used to reduce the burden on providers for reporting quality performance data to CMS? What capabilities are needed to show benefit? What concerns are there with this approach?

- By enabling bulk FHIR data exports directly from the EHR to CMS, providers no longer need to manually abstract data, use separate reporting tools, or submit fragmented files. CMS can then perform the quality metric calculations, significantly shifting the burden facing the physician community.

TD–11. As of January 1, 2024, many health IT developers with products certified through the ONC Health IT Certification Program are required to include the capability to perform an electronic health information export or ‘‘EHI export’’ for a single patient as well as for patient populations (45 CFR 170.315(b)(10)). Such health IT developers are also required to publicly describe the format of the EHI export. Notably, how EHI export was accomplished was left entirely to the health IT developer. Now that this capability has been in production for over a year, CMS and ASTP/ONC seek input on the following:

a. Should this capability be revised to specify standardized API requirements for EHI export?

Yes, Cohere believes this capability should be revised to specify standardized API requirements for EHI support. Leaving the EHI export format entirely to the health IT developer creates a construct, where each export is potentially different, leading to significant burden on the receiving end to parse and integrate the data. Specifying standardized API requirements (e.g., using Bulk FHIR for population export and FHIR for single patient export) is crucial for true interoperability and usability.

4. Data Exchange

TD–12. Should CMS endorse non-CMS data sources and networks, and if so, what criteria or metrics should CMS consider?

Yes, CMS should endorse non-CMS data sources and networks that meet rigorous criteria. The healthcare ecosystem is vast, and limiting interoperability to CMS-controlled data would severely hinder comprehensive patient care. Criteria/Metrics:

- Adherence to Standards: Strict compliance with USCDI and FHIR APIs for data exchange.
- Security and Privacy: Demonstrated robust security practices and full compliance with HIPAA and other privacy regulations.
- Interoperability Performance: Measurable metrics on data exchange success rates, timeliness, and completeness.
- Patient Matching Accuracy: High accuracy in patient identity matching.

- Network Reach and Participation: Breadth of network coverage and active participation from diverse healthcare organizations.
- Governance and Trust Framework: Clear and transparent governance structures that promote trust and accountability.
- Auditing and Accountability: Mechanisms for auditing compliance and addressing information blocking or data quality issues.
- Value to Patients/Providers/Payers: Demonstrated ability to improve patient care, reduce provider burden, or streamline payer operations .

TD–13. What new opportunities and advancements could emerge with APIs providing access to the entirety of a patient's electronic health information (EHI)?

Cohere believes that access to the entirety of a patient's EHI via APIs would unlock transformative opportunities, as Cohere has seen firsthand with the power of our own predictive models.

- True AI-Driven Clinical Decision Support: AI/ML models can analyze a complete patient history (including unstructured notes, faxes) to provide highly accurate diagnoses, personalized treatment recommendations, and identify complex drug interactions or contraindications. Cohere is able to achieve real-time prior authorization decisions with the data we ingest today, however, the clinical decision support capabilities that can be unlocked by providing access to the entirety of the EHI are significant.
- Proactive Disease Management & Personalized Care Paths: Identify patients at high risk for specific conditions and intervene proactively and Develop highly customized care plans that adapt in real-time to a patient's evolving health status and preferences.
- Comprehensive Prior Authorization Automation: Automate complex PA requests by pulling all necessary documentation directly from the EHR, enabling real-time approvals for most cases and only sending complex ones for physician review. Again, this is core to Cohere Health's core offering, however, having true outcome data and information on total cost of care will significantly enhance these models.
- Seamless Care Transitions: Ensure complete patient data follows them through every care setting, reducing errors and improving outcomes.
- Accelerated Clinical Research: Researchers would have access to vastly richer datasets for studies, leading to faster discovery of new treatments and therapies.
- Enhanced Public Health Surveillance: More robust real-time data for tracking disease outbreaks and population health trends.
- What are the primary obstacles to this?
    - Information Blocking: The most significant obstacle is information blocking as many EHR vendors and healthcare organizations still resist open access to the full EHI.
    - Legacy Systems and Unstructured Data: The challenge of converting vast amounts of unstructured and legacy data into usable, API-accessible formats.
    - Privacy and Security Concerns: Ensuring robust safeguards for such a large and sensitive dataset.
    - Patient Matching: Accurately linking all pieces of EHI to the correct patient across disparate systems.
    - Cost and Complexity of Integration: The immense effort required to integrate and normalize data from countless sources.
    - Lack of Clear Legal Precedent/Enforcement: Uncertainty and inconsistent enforcement around data access rights.

TD–14. Regarding networks' use of FHIR APIs: a. How many endpoints is your network connected to for patient data sharing? What types, categories, geographies of endpoints do you cover? Are they searchable by National Provider Identifier (NPI) or organizational ID?

Cohere Health, as an intelligent prior authorization platform, connects to numerous endpoints primarily through integrations with payers (Medicare Advantage, Commercial, Managed Medicaid plans) and providers (EHRs, practice management systems). Our focus is on the efficient, real-time exchange of data for authorization requests and decisions, which often involves patient clinical data.

- Number of Endpoints: This number is dynamic and grows as we integrate with more plans and provider systems. We don't maintain a public count of individual FHIR endpoints we connect to, as our integrations are often customized. However, we process millions of authorization requests annually, indicating a broad reach.
- Types/Categories/Geographies: Our connections span payer systems (for eligibility, benefits, and claims data), provider EHRs (for clinical documentation), and our own intelligent authorization platform. Geographically, we cover clients and providers across all 50 states where our payer partners and risk-bearing providers operate.
- Searchable by NPI or Organizational ID: Yes, our internal systems and integration points are designed to identify providers by NPI and organizational ID to ensure accurate routing of authorization requests and associated clinical data. This is fundamental for our prior authorization workflows.

b. How are these connections established (for example, FHIR (g)(10) endpoints, TEFCA/Integrating the Health Enterprise (IHE) XCA, or proprietary APIs)?

Cohere's connections are established through a combination of approaches, including:

- FHIR APIs: We actively leverage FHIR APIs where available and robustly implemented by our partners, particularly for patient data access and (increasingly) for prior authorization (e.g., Da Vinci CRD/DTR/PAS). We strongly advocate for their universal adoption.
- Proprietary APIs: Given the current reality, we often develop custom integrations with proprietary APIs offered by EHRs and payer systems. This allows us to connect with a wider range of partners and serve their specific needs.
- Web Services/Other Standards: Employ various web services and industry standards for data exchange, adapting to our partners' capabilities.
- Fax Conversion: Given that 50% of PA submissions are still fax-based, we also utilize advanced fax conversion capabilities to digitize and ingest these requests, enabling downstream intelligent processing. Our nearly 90% portal adoption rate for providers demonstrates our ability to transition them to digital.

c. Do you interconnect with other networks? Under what frameworks (for example, TEFCA, private agreements)?

We currently interconnect with payers and providers primarily through private agreements that define the scope and security of data exchange. As a healthcare technology provider, our direct interconnection is with our customers (payers) and their contracted providers. While we are not a QHIN, Cohere Health sees significant potential in TEFCA. As TEFCA matures and QHINs become more widespread, we would be keen to integrate with QHINs to facilitate broader, more standardized data exchange for prior

authorization and related clinical data, under the TEFCA framework. Our platform is built to be interoperable and can adapt to new frameworks.

TD–15. Regarding bulk FHIR APIs: a. How would increased use of bulk FHIR improve use cases and data flow?

- Quality Reporting: Dramatically simplify and automate the process of collecting quality performance data for CMS and payers, reducing provider burden.
- Population Health Management: Enable comprehensive analysis of patient populations for risk stratification, care gap identification, and proactive interventions.
- Clinical Research: Facilitate large-scale data extraction for research, accelerating medical discovery.
- AI Model Training: Provide rich, de-identified datasets for training and validating advanced AI/ML models for various healthcare applications, including prior authorization appropriateness.
- Care Management: Allow care managers to pull complete patient panels to identify patients needing specific interventions or outreach.
- Interoperability for New Entrants: Lower the barrier for new technology vendors to integrate and access large datasets.

b. What are the potential disadvantages of their use?

- Data Volume and Processing: The sheer volume of data can pose significant storage and processing challenges for receiving systems.
- Privacy and Security Concerns: Managing and de-identifying large datasets requires robust security measures and careful adherence to privacy regulations.
- Data Quality and Consistency: Ensuring data consistency and accuracy when pulling bulk data from diverse sources remains a challenge.
- Infrastructure Costs: Significant investment required for robust infrastructure to handle and transfer bulk data.
- Ethical Use: Ensuring that large datasets are used ethically and do not perpetuate biases.

TD–17. Given operational costs, what role should CMS or ASTP/ONC or both have in ensuring viability of healthcare data sharing networks, including enough supply and demand, that results in usage and outcomes?

CMS and ASTP/ONC have a critical role in ensuring the viability of healthcare data sharing networks. Without their active involvement, the market alone may not overcome the inherent challenges (fragmentation, information blocking, high upfront costs).

5. Compliance

TD–18. Information blocking:

a. Could you, as a technology vendor, provide examples for the types of practices you have experienced that may constitute information blocking. Please include both situations of non-responsiveness as well as situations that may cause a failure or unusable response?

As Cohere Health focuses on streamlining prior authorization, we regularly encounter practices that impede efficient data exchange, often falling under the spirit, if not the letter, of information blocking. We most commonly see vendors pushing proprietary APIs that require custom integrations and charge

effectively making open, standards-based API use "not practicable" and generally lacking simple designs and interactiveness to connect with smaller systems, which is critical for adoption and true interoperability.

b. What additional policies could ASTP/ONC and CMS implement to further discourage healthcare providers from engaging in information blocking practices?

- Increase Penalties: Implement more significant and direct financial penalties for all categories of actors found to be information blocking, consistent with the 21st Century Cures Act.
- Tie to Payment/Performance: Link compliance with information blocking rules directly to participation in Medicare payment programs, quality incentives, or value-based care models.
- Public Transparency: Publicly name entities found to be information blocking.
- Streamlined Complaint Process: Make it easier and safer for providers and technology vendors to report information blocking without fear of retaliation.
- Proactive Audits: Conduct regular, proactive audits of health IT developers and providers to ensure compliance, rather than relying solely on reactive complaints.
- Define "Special Effort" & "Interference" More Clearly: Provide more specific examples of what constitutes "special effort" or "interference" that is not permissible, leaving less room for misinterpretation.
- Mandate API Performance Metrics: Require certified health IT to report on API performance (e.g., response times, uptime, data completeness) to ensure usability.

c. Are there specific categories of healthcare actors covered under the definition of information blocking in section 3022(a)(1) of the Public Health Service Act (PHSA) that lack information blocking disincentives?

Cohere believes transparency and data sharing are key pillars to support the health care delivery system of the future. While the PHSA broadly defines "healthcare actor," the direct disincentives established by CMS to date are primarily focused on health IT developers and some healthcare providers. It is critical to ensure that all categories of actors (including *all* types of providers, health information exchanges, and health information networks) have clear and meaningful disincentives for information blocking. This includes payers and other data fiduciaries if their practices impede EHI access, exchange, or use. Without universal disincentives, loopholes will persist, and the goal of seamless interoperability will remain elusive.

TD–19. Regarding price transparency implementation: a. What are current shortcomings in content, format, delivery, and timeliness?

b. Which workflows would benefit most from functional price transparency?

- Prior Authorization: Integrating cost estimates directly into the prior authorization process would allow patients and providers to make more informed decisions about care, considering both clinical appropriateness and financial implications.
- Patient Financial Counseling: Empowering patients to understand their financial responsibility upfront.
- Care Planning: Allowing providers to consider cost alongside clinical effectiveness when recommending treatment options.
- Referral Management: Helping patients choose cost-effective options within their network.
- Population Health Management: Identifying opportunities for cost savings at a population level.

c. What improvements would be most valuable for patients, providers, or payers, including CMS?

- Personalized Estimates: For patients, reliable, binding estimates of their actual out-of-pocket costs before receiving care.
- Standardized API for Price Transparency: For providers and payers, a common API to query and provide accurate price information, facilitating integration into workflows.
- Bundled Service Pricing: For all stakeholders, transparency on the cost of an entire episode of care, not just individual line items.
- Integration with Prior Authorization: For payers, a mechanism to provide price transparency alongside prior authorization decisions, ensuring appropriate and cost-effective care.

d. What would further motivate solution development?

- Clear Mandates for API-driven Price Transparency: Requiring hospitals, payers, and providers to expose binding price estimates via standardized APIs.
- Enforcement of Transparency Rules: Penalties for non-compliance.
- Incentives for Innovation: Funding or grants for developers building price transparency tools.
- Integration with Value-Based Care: Tying price transparency metrics to VBC performance.

F. Value-Based Care Organizations

Prior authorization data and the insights gleaned from its deployment can be leveraged to develop metrics concerning the appropriateness of care within total cost of care models, empowering CMS and payers to effectively monitor and, as appropriate, reward or penalize physicians. For example, intelligent PA can predict physician behavior and proactively recommend timely services (e.g., an authorization for surgery could trigger the timely arrangement of home-based post-acute care or the delivery of durable medical equipment [DME]), or suggest shifts in the site of service (e.g., from inpatient to outpatient/ambulatory surgery centers when clinically appropriate), which are significantly more cost effective.